

Cardiac Tamponade		
Condition/Cause	Treatment	Monitor
Cardiac tamponade is the accumulation of fluid around the heart, often due to pericarditis with bacteria-containing fluid or chest injury, which increases pressure on the heart and impairs its function, with the pericardial space normally containing lubricating fluid for smooth heart movement.	1. Pericardiocentesis: • A procedure to remove excess fluid from the pericardial sac using a needle. • Has both therapeutic and diagnostic effects. The fluid is cultured to identify bacterial or microbial presence and antibiotic sensitivity, guiding appropriate treatment. 2. Fluid Resuscitation: • Intravenous fluids to maintain blood pressure and cardiac output. • Hydrate the patient before any fluid aspiration to prevent hypotension. 3. Treatment of Underlying Cause: • Address the root cause, such as infection, cancer, or autoimmune disease. 4. Emergency Action: • Since cardiac tamponade is life-threatening, call a physician immediately. • Cardiac tamponade requires immediate medical attention to prevent severe complications or death. Seek emergency medical care if suspected.	1. EKG strip with one QRS complex up and one down for monitoring. 2. Any fluid aspiration can lead to hypotension; ensure the patient is well-hydrated before the procedure.
Symptoms Sharp Chest Pain: Pain during expiration or coughing. Orthopnea: Difficulty breathing when lying flat; a supine position is not recommended. SOB: Difficulty breathing even at rest or with minimal exertion. Beck's Triad (Hallmark Symptoms): • Hypotension • Jugular Venous Distension (JVD) • Muffled or Diminished Heart Tones Pulsus Paradoxus: An exaggerated decrease in systolic blood pressure during inspiration, where the systolic pressure drops significantly while diastolic pressure remains the same, causing the pressures to get closer to each other.		Patient Assessment Questions to ask the patient: • Does the pain radiate? • Is the pain above 8 on the pain scale? • Does your cough produce blood-tinged sputum? Note: Myocardial infarction (MI) does not typically present with a cough.
		PULSUS PARADOXUS Systolic drop by 10 mmHg upon respiration Narrowed pulse pressure 120 / 80 - Normal pressure 109 / 80 - 11 pulsus Paradoxus!